greenlight by Medivis

Business Proposal & Market Opportunity

AI-Powered Prior Authorization for Imaging, Surgery & Procedures

$35B	25-30%	200M+	$7.1B+
PA ADMIN SPEND	IMAGING DENIAL RATE	ANNUAL PA REQUESTS	REVENUE TAM

Prepared: March 2026

Classification: Confidential — Version 2.0

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SECTION 01

Executive Summary

Core Thesis: Prior authorization is the single largest administrative burden in healthcare, costing $35 billion annually. Imaging and procedures face the worst of it — 25-30% denial rates, 91% of radiology encounters requiring PA, and $2M+ in annual revenue leakage per imaging center. Yet no dominant vertical solution exists for this segment. GreenLight by Medivis automates the entire PA lifecycle for imaging, surgery, and procedures — reducing processing time from days to seconds, cutting denials by 75%, and recovering millions in lost revenue.

$35B
ANNUAL PA ADMIN COSTS

91%
RADIOLOGY ENCOUNTERS REQUIRING PA

200M+
PA REQUESTS PROCESSED ANNUALLY (US)

$7.1B+
GREENLIGHT BY MEDIVIS REVENUE TAM

What We're Building

GreenLight by Medivis is an AI-native platform purpose-built to automate prior authorization for imaging (MRI, CT, PET, ultrasound), surgical procedures, and medical procedures. Unlike horizontal PA solutions that treat all service types the same, GreenLight is specialized for the unique complexities of imaging and procedural PA — including RBM navigation (EviCore, CareIon, NIA), ACR Appropriateness Criteria matching, PACS/RIS integration, and surgical scheduling coordination.

Business Model

Per-transaction pricing: **$8** per automated PA. A hospital processing **15,000** PAs/month pays **$1.44M**/year while saving **$2M+** in staff costs and **$1M+** in recovered revenue from prevented denials. Performance guarantee: if GreenLight by Medivis doesn't deliver measurable savings, clients pay nothing.

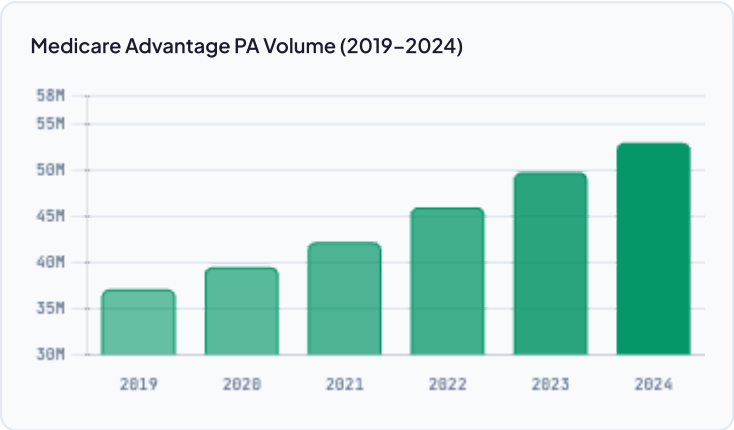
5-Year Trajectory

	YEAR 1	YEAR 2	YEAR 3	YEAR 4	YEAR 5
ARR	$2.1M	$12.5M	$42M	$108M	$245M
Customers	15	65	180	420	850
PA Volume/Month	45K	260K	875K	2.25M	5.1M
Exit Valuation	—	—	Series B	IPO-ready	$3-5B

SECTION 02

Market Problem: The Prior Authorization Crisis

Prior authorization has become the most hated process in healthcare. It delays care, increases costs, and drives clinician burnout at unprecedented rates.



$82.7B
TOTAL MEDICAL ADMIN TRANSACTION COSTS (2023), UP 50% FROM $55B IN 2022

39-45
PRIOR AUTHS PER PHYSICIAN PER WEEK

16.4 hrs/wk
STAFF TIME ON PA PER PHYSICIAN

94%
OF PHYSICIANS SAY PA DELAYS CARE

$11K
PER CLINICIAN/YEAR DIRECT PA COSTS

$118
PER DENIED CLAIM REWORK EXPENSE

78%
PATIENTS ABANDON CARE DUE TO PA DELAYS

The system is broken: 200+ million PA requests per year. Average turnaround of 5 days. 25-30% denial rates for imaging. 80.7% of Medicare Advantage appeals are overturned — proving most denials are inappropriate. Physicians report that prior authorization is the single largest driver of burnout.

SECTION 03

Imaging, Surgery & Procedures: The Unserved Niche

While pharmacy PA is largely solved (SureScripts processes 30.5B transactions with 18-second approvals), medical PA for imaging and procedures remains stuck in the dark ages. This is the whitespace.

Why This Niche?

The Burden

- Diagnostic radiology: **91%** of encounters require PA
- Radiation oncology: **97%**
- Cardiology: **93%**
- Neurosurgery: **90%**
- Imaging denial rates: **25-30%** vs **7.7%** overall MA average
- MRI spine denials exceed **30%** for some insurers

The Complexity

- Multiple RBMs (EviCore, Carelon, NIA) with different portals and rules
- Payers switch RBMs without notice
- Clinical criteria matching (ACR, MCG, InterQual)
- PACS/RIS integration requirements
- Multi-step procedure coordination (biopsy after imaging)
- Site-of-service optimization

Revenue Impact by Facility Type

FACILITY TYPE	ANNUAL PA VOL	DENIAL RATE	REVENUE AT RISK	ANNUAL LEAKAGE
Outpatient Imaging Center	8,000-12,000	25-30%	$12-15M	$1.5-2M
Ambulatory Surgery Center	3,000-6,000	18-22%	$15-25M	$1-2.5M
Hospital Radiology Dept	15,000-30,000	20-28%	$30-60M	$3-8M
Multi-Hospital System	50,000-150,000	22-28%	$100-300M	$10-30M

The key insight: No dominant vertical solution exists for imaging and procedural PA. Cohere Health is payer-side and horizontal. Waystar is provider-side but horizontal. SureScripts dominates pharmacy but not medical PA. EviCore, Carelon, and NIA are the problem, not the solution. GreenLight by Medivis fills this whitespace with a purpose-built platform.

SECTION 04

Product: The GreenLight by Medivis Platform

Six integrated capabilities that automate the entire prior authorization lifecycle for imaging, surgery, and procedures.

1. Intelligent Submission

Auto-builds PA requests with complete clinical documentation. Matches indications to payer-specific guidelines. Reduces submission time from 20 minutes to 45 seconds.

20 min → 45 sec | Auto-documentation

2. Real-Time Eligibility

PA requirement detection at point of order. FHIR CRD integration checks coverage requirements before the clinician finishes placing the order.

600+ payer rules | Point-of-order

3. Denial Prevention

Pre-submission AI audit against historical denial patterns. Flags missing documentation, incorrect codes, and guideline mismatches before the PA is sent.

75% denial reduction | Pre-submission

4. Appeal Automation

Auto-generates peer-to-peer prep packages and formal appeal letters with clinical evidence. Leverages 80.7% MA appeal overturn rate.

80.7% win rate | Auto-escalation

5. Unified Dashboard

Single view across all payers and RBMs. Real-time status tracking, analytics, and bottleneck identification across the entire PA portfolio.

Multi-RBM | Real-time status

6. Scheduling Sync

Connects PA status directly to imaging and OR scheduling systems. Eliminates scheduling conflicts from pending authorizations.

Zero conflicts | OR/imaging sync

End-to-end automation: GreenLight by Medivis handles the full PA lifecycle — from requirement detection at point of order, through intelligent submission, real-time tracking, denial prevention, appeal automation, and scheduling coordination. No manual handoffs. No portal-hopping. No fax machines.

SECTION 05

Total Addressable Market

A massive, underserved market with clear paths to $245M ARR within five years.

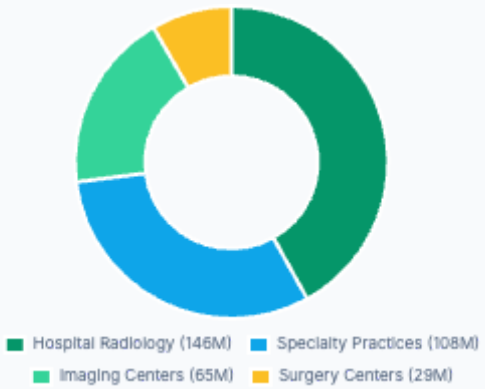
TAM / SAM / SOM

TAM
$7.1B+
$35B TOTAL PA ADMIN SPEND. $8/TRANSACTION ON 200M+ TRANSACTIONS.

SAM
$3.2B
IMAGING, SURGERY, PROCEDURES = ~45% OF ALL MEDICAL PAS

SOM (YEAR 5)
$245M
5.1M PAS/MONTH × $6 AVG × 12 MONTHS TARGETING

Market Distribution by Facility Type



US Healthcare Facilities — Imaging & Procedure PA Market

FACILITY TYPE	COUNT	AVG MONTHLY PA VOL	TOTAL MARKET PAS/YEAR
Freestanding Imaging Centers	6,800+	800	65M
Ambulatory Surgery Centers	6,100+	400	29M
Hospital Radiology Depts	6,090	2,000	146M
Specialty Practices (Cardio, Neuro, Ortho)	45,000+	200	108M
Total			**348M**

SECTION 06

Competitive Landscape

The PA automation market is fragmented and horizontal. No player dominates the imaging and procedure vertical.

Tier Analysis

COMPANY	FOCUS	TYPE	FUNDING	WEAKNESS
Cohere Health	All PA (payer-side)	Horizontal	$200M	Payer-side; not imaging-specific
Waystar	Revenue cycle + PA	Horizontal	Public ($4.7B)	Broad RCM, PA is secondary
Availity	Clearinghouse + PA	Network	Private	Portal-based, not AI-native
Infinitus	Voice AI for PA calls	Point solution	$103M	Follow-up only, not end-to-end
Myndshft/DrFirst	Real-time PA	Horizontal	Acquired	Generic rules, not imaging-specialized
EviCore	Radiology benefit mgmt	RBM (incumbent)	Evernorth/Cigna	IS the bottleneck, not the solution
Carelon	Specialty benefit mgmt	RBM (incumbent)	Elevance	Same — creates the problem
SureScripts	Pharmacy PA	Pharmacy only	Private	Does not serve medical/imaging PA

GreenLight by Medivis Differentiation:

1. Imaging-native intelligence: Purpose-built for imaging modalities and surgical procedures, not a generic PA workflow tool

2. RBM navigation layer: Automates interactions with EviCore, Carelon, NIA — the specific bottleneck for imaging

3. ACR criteria matching: Built-in clinical decision support using ACR Appropriateness Criteria

4. PACS/RIS integration: Connects to imaging-specific systems, not just EHR

5. Scheduling coordination: Links PA status directly to OR and imaging appointment workflows

6. Per-transaction pricing: Transparent $8/PA vs. opaque PMPM or enterprise contracts

SECTION 07

Business Model & Pricing

A transparent, value-aligned pricing model that scales with customer success.

Per-Transaction Model

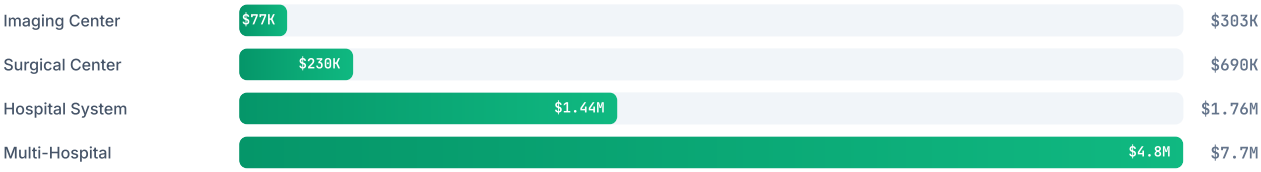
- ✓ **$8** per automated PA transaction
- ✓ Performance guarantee: **$0** if no measurable savings
- ✓ No upfront costs, no annual contracts required
- ✓ Volume discounts for **10,000+** PAs/month

Why per-transaction wins: Unlike PMPM pricing, per-transaction aligns cost directly with value. Clients pay only for PAs we actually process. No shelf-ware risk. No wasted spend on unused licenses. The more value GreenLight by Medivis delivers, the more both parties benefit.

Example Scenarios

SCENARIO	MONTHLY VOL	ANNUAL COST	SAVINGS GENERATED	NET BENEFIT	ROI
Imaging Center (5 radiologists)	800	$76,800	$380,000	**$303,200**	**3.9x**
Surgical Center (15 surgeons)	2,400	$230,400	$920,000	**$689,600**	**3.0x**
Hospital System (100 providers)	15,000	$1,440,000	$3,200,000	**$1,760,000**	**1.2x**
Multi-Hospital Network (500 providers)	50,000	$4,800,000	$12,500,000	**$7,700,000**	**1.6x**

Revenue Scaling Visualization



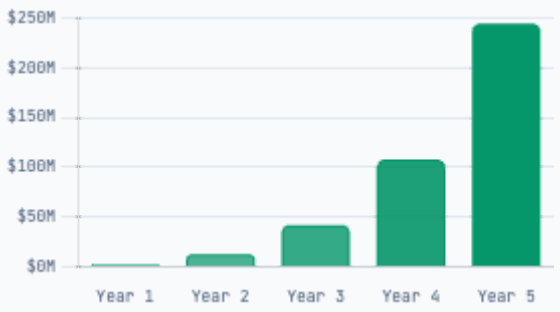
SECTION 08

Financial Model: 5-Year ARR Projections

METRIC	YEAR 1	YEAR 2	YEAR 3	YEAR 4	YEAR 5
New Customers	15	50	115	240	430
Total Customers	15	65	180	420	850
Avg Monthly PA/Customer	3,000	4,000	4,860	5,360	6,000
Total Monthly PA Volume	45,000	260,000	875,000	2,250,000	5,100,000
Revenue/PA	$8.00	$7.50	$7.00	$6.50	$6.00
Monthly Revenue	$360K	$1.95M	$6.13M	$14.63M	$30.6M
ARR	**$2.1M***	**$12.5M**	**$42M**	**$108M**	**$245M**
Gross Margin	65%	72%	78%	82%	85%
Net Revenue Retention	120%	130%	135%	130%	125%

*Year 1 ARR is annualized run-rate at month 12

ARR Growth (Year 1–5)



Unit Economics

METRIC	YEAR 1	YEAR 5
CAC	$45,000	$25,000
ACV	$140,000	$288,000
LTV/CAC Ratio	3.1x	11.5x
Payback Period	4 months	1.5 months
Gross Margin	65%	85%

$245M

Year 5 ARR Target — 850 Customers — 5.1M Monthly PAs

SECTION 09

Implementation Strategy & Timeline

Phase 1: Foundation (Months 1–6)

- Build core PA submission engine
- Integrate FHIR CRD/DTR/PAS specifications
- Connect to top 5 payers (UHC, Anthem, Aetna, Cigna, Humana)
- Integrate with top 3 RBMs (EviCore, Carelon, NIA)
- Deploy at 3–5 beta customers (imaging centers)

◆ **Milestone: First fully automated PA submission**

Phase 2: Expansion (Months 7–12)

- Expand payer/RBM coverage to top 20 payers (85% market coverage)
- Add denial prevention AI engine
- Build appeal automation workflow
- Epic FHIR integration
- Launch scheduling sync module
- Scale to 15 customers

◆ **Milestone: $2.1M ARR run-rate**

Phase 3: Scale (Months 13–24)

- Add surgical center and hospital system modules
- Build PACS/RIS direct integration
- Expand to 65 customers
- Launch self-service onboarding for smaller facilities

◆ **Milestone: $12.5M ARR**

Phase 4: Market Leadership (Months 25–36)

- National payer coverage (95%+ of commercial lives)
- Launch payer-side analytics product (secondary revenue stream)
- Strategic partnerships with EHR vendors

◆ **Milestone: $42M ARR, Series B raise**

Development Gantt Chart

SECTION 10

Go-to-Market Strategy

Land in imaging centers, expand to surgical centers and hospital systems, then capture the long tail of specialty practices.

Target Segments (Prioritized)

PRIORITY 1

Freestanding Imaging Centers

6,800+ in US. Highest pain, fastest decisions, highest imaging PA volume per provider.

PRIORITY 2

Ambulatory Surgery Centers

6,100+. High-value procedures, PA delays directly impact OR utilization.

PRIORITY 3

Hospital Radiology Departments

6,090. Largest volume, longest sales cycle, highest ACV.

PRIORITY 4

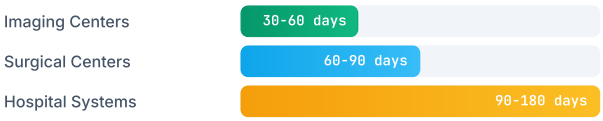
Specialty Practices

Cardiology, orthopedics, neurosurgery. High PA burden specialties.

GTM Motions

- **Outbound:** Target imaging center administrators and surgical center directors via personalized ROI analyses
- **Content marketing:** PA burden calculators, denial rate benchmarks, regulatory updates (CMS-0057)
- **Channel partnerships:** EHR vendors, clearinghouses, radiology groups (RadPartners, SimonMed, Akumin)
- **Conference strategy:** RSNA, RBMA, ASC Association, HIMSS
- **Referral program:** $5,000 per referred customer

Sales Cycle by Segment



Land-and-expand strategy: Start with a single department or location, prove ROI within 60 days, then expand across the health system. Average expansion from 1 to 4+ locations within 12 months.

Year 1 GTM Timeline

Q1
Beta launch, 3-5 customers, product-market fit validation

Q2
First paid customers, case studies, initial conference presence

Q3
Scale outbound, channel partner recruitment, 10+ customers

Q4
15 customers, $2.1M ARR run-rate, Series A preparation

SECTION 11

Risk Analysis & Mitigation

A clear-eyed assessment of risks and our strategies to address each one.

RISK	PROBABILITY	IMPACT	MITIGATION
Payer portal changes break automation	High	Medium	Multi-method integration (API, FHIR, portal, fax); dedicated monitoring and rapid adaptation team
CMS-0057 reduces PA burden overall	Medium	Medium	Electronic mandate creates more automation opportunities, not fewer; positions GreenLight by Medivis as compliance solution
EHR vendors build native PA tools	Medium	High	Specialize deeper into imaging/procedure niche; EHR tools will be generic; partner vs. compete
Large horizontal competitor enters niche	Medium	Medium	First-mover advantage in imaging-specific intelligence; high switch costs once integrated with PACS/RIS
Customer concentration in early years	High	High	Diversify across facility types; enforce no single customer > 15% of revenue; rapid customer acquisition
HIPAA/security incident	Low	Critical	SOC 2 Type II, HIPAA BAA, zero-retention LLM architecture, regular penetration testing, dedicated security team

Risk Heatmap

	Low Impact	Medium Impact	High/Critical
High Prob		Portal changes	Customer conc.
Med Prob		CMS-0057; Competitor	EHR native tools
Low Prob			HIPAA incident

Regulatory Tailwinds

CMS-0057 Final Rule (Jan 2026): Requires payers to implement electronic PA APIs. This is a massive tailwind — it mandates the infrastructure GreenLight by Medivis connects to, while still requiring specialized provider-side automation to fully leverage.

State-level PA reform: 30+ states have enacted or proposed PA reform legislation. Texas, Michigan, and Florida now require 48-hour turnaround for urgent PAs. Regulatory pressure is accelerating demand for automation.

SECTION 12

Technical Architecture

Enterprise-grade, HIPAA-compliant, built for healthcare from day one.

Architecture Components

COMPONENT	TECHNOLOGY	WHY
AI/LLM Backend	LLM via AWS Bedrock	BAA-eligible, HIPAA-compliant, zero data retention
Agent Orchestration	LangGraph	Graph-based, stateful, auditable PA workflows
Durable Execution	Temporal	Guarantees PA completion despite failures
Integration Engine	Mirth Connect	FHIR, HL7v2, X12 EDI translation
Payer Connectivity	Availity + Direct FHIR	Clearinghouse + Da Vinci PAS
EHR Integration	Epic FHIR, Oracle Health	CRD/DTR at point of order
Imaging Integration	PACS/RIS APIs	Direct radiology workflow connection
Infrastructure	AWS (GovCloud eligible)	FedRAMP path for government contracts

FHIR Da Vinci Standards

CRD — Coverage Requirements Discovery
Real-time "is PA required?" check at point of order. Intercepts the clinical workflow before time is wasted.

DTR — Documentation Templates & Rules
Auto-populate clinical data from EHR. Reduces manual data entry to near zero.

PAS — Prior Authorization Support
Submit and receive PA decisions electronically. Replaces fax, phone, and portal submissions.

Security & Compliance

- ✓ HIPAA BAA with all subprocessors
- ✓ SOC 2 Type II certification (target: Month 8)
- ✓ Zero-retention LLM architecture (no PHI stored in model)
- ✓ End-to-end encryption (TLS 1.3 in transit, AES-256 at rest)
- ✓ Role-based access control (RBAC)
- ✓ Audit logging for all PA transactions
- ✓ Regular penetration testing (quarterly)
- ✓ 99.95% uptime SLA

99.95%
UPTIME SLA

<200ms
API RESPONSE TIME

SECTION 13

Clinical Data Strategy: Payer Rules & Criteria Sources

The clinical rules engine is GreenLight's core IP. This section maps the authoritative data sources that power payer-specific PA determination, denial prevention, and appeal generation.

Tiered Data Acquisition Strategy

Tier 1 — Public Clinical Standards (Free)

Foundational clinical correctness layer. Available immediately, no licensing required.

- **ACR Appropriateness Criteria:** ~200+ clinical conditions mapped to imaging modalities. Each topic rated 1–9 (usually appropriate / may be appropriate / usually not appropriate). Machine-readable via ACR API at acsearch.acr.org
- **CMS NCDs & LCDs:** National & Local Coverage Determinations define exactly what Medicare covers and what requires PA. Published in structured formats at cms.gov
- **CMS Annual PA List:** Medicare Advantage prior authorization requirements published annually — definitive source for MA plans

No cost | Immediate access | Machine-readable

Tier 2 — RBM Published Guidelines (Free)

The actual criteria that determine approval/denial for most commercial plans. Published as PDFs, parseable with AI.

- **EviCore** (Evernorth/Cigna): Clinical guidelines at evicore.com/provider, organized by specialty — radiology, cardiology, oncology, musculoskeletal
- **Carelon** (Elevance/Anthem): Clinical appropriateness guidelines at carelonmedicalbenefits.com — covers imaging, radiation therapy, sleep studies
- **NIA / Evolent Health:** National Imaging Associates guidelines on provider portal — primarily advanced imaging (MRI, CT, PET, nuclear medicine)

No cost | PDF parsing required | Highest denial-prevention value

Tier 3 — Payer-Specific Policies (Free, Manual)

Individual payer PA requirements by CPT code, plan type, and state. Scattered across provider portals.

- **UnitedHealthcare:** uhcprovider.com → Prior Authorization & Notification lookup tool
- **Aetna:** aetna.com/health-care-professionals → Precertification lists by plan
- **Cigna:** cignaforhcp.cigna.com → Coverage Policies searchable by CPT
- **BCBS:** Varies by state plan — each publishes a medical policy manual with PA requirements
- **Humana, Anthem, Centene:** Provider portals with PA lookup tools

No cost | High maintenance | Changes without notice

Tier 4 — Licensed Clinical Criteria (Commercial)

Industry-standard clinical decision support used by payers themselves to adjudicate PA requests.

- **InterQual (Cotiviti):** Industry-standard criteria used by major payers. Structured, machine-readable criteria sets. Covers imaging appropriateness, surgical necessity, and level-of-care
- **MCG (Milliman Care Guidelines):** Used by ~2,700 hospitals and health plans. Covers imaging, surgical, and procedure appropriateness with evidence-based indications

Commercial license | Authoritative | Machine-readable

SECTION 13 (CONT.)

Clinical Data Strategy: Implementation Roadmap

Data Source Coverage Matrix

SOURCE	COVERAGE	FORMAT	COST	UPDATE FREQUENCY	INTEGRATION PRIORITY
ACR Appropriateness Criteria	~200 clinical topics, all imaging modalities	API + structured data	Free	Annual	P0 — Launch
CMS NCDs / LCDs	All Medicare-covered services	Structured database	Free	Quarterly	P0 — Launch
EviCore Clinical Guidelines	Radiology, cardio, oncology, MSK	PDF (AI-parseable)	Free	Semi-annual	P0 — Launch
Carelon Guidelines	Imaging, radiation, sleep	PDF (AI-parseable)	Free	Semi-annual	P1 — Month 3
NIA / Evolent Guidelines	Advanced imaging (MRI, CT, PET)	PDF	Free	Annual	P1 — Month 3
Payer PA Requirement Lists	CPT-level PA requirements by plan	Portal / PDF / web scrape	Free	Monthly (changes without notice)	P1 — Month 3
CAQH CORE / X12 278	Electronic PA eligibility responses	EDI / API	Partnership	Real-time	P2 — Month 6
Availity / Change Healthcare	PA requirement data-as-a-service	API	Commercial	Real-time	P2 — Month 6
InterQual (Cotiviti)	Full clinical decision support	API / structured	$$$ License	Quarterly	P3 — Month 9
MCG (Milliman)	Full clinical decision support	API / structured	$$$ License	Quarterly	P3 — Month 9

Why This Is the Moat

The rules engine is the product. The platform UI and workflow automation are table stakes. The real value is knowing — for a specific payer, plan, state, CPT code, and clinical scenario — exactly what documentation is needed, what criteria must be met, and what language will get an approval. This knowledge compound over time as GreenLight processes more PAs and learns from outcomes.

Keeping rules current is the moat. Payers change PA requirements frequently and often without notice. EviCore updates clinical guidelines semi-annually. State Medicaid programs change quarterly. Building a system that continuously monitors, parses, and ingests these changes — before they cause denials — is what separates GreenLight from horizontal PA tools.

Rules Engine Flywheel

1. Ingest
Parse ACR, RBM guidelines, payer policies

↓

2. Match
Map clinical scenario to payer-specific criteria

↓

3. Submit
Generate optimized PA with complete documentation

↓

4. Learn
Track outcomes, refine criteria matching, identify rule changes

↑ ↻

APPENDIX

Platform Naming Considerations

Five naming options evaluated for market positioning, memorability, and brand extensibility.

1. GreenLight by Medivis

RECOMMENDED

"Get the green light for every authorization."

Pros: Intuitive metaphor — "green light" = approval. Works as a verb ("GreenLight your MRI"). Memorable and distinctive. Strong brand color opportunity.

Risks: May feel too casual for enterprise healthcare. Common phrase could present trademark challenges.

2. ClearPath Health

"Clearing the path through authorization."

Pros: Professional, implies efficiency and obstacle removal. Easy to understand. Works well for enterprise sales.

Risks: Generic sounding. Multiple companies use "ClearPath" across industries. Less distinctive.

3. Aperture Health

"Precision focus on authorization."

Pros: Aperture = opening in optics/imaging. Perfect imaging reference. Sophisticated and unique. Strong visual brand potential.

Risks: Portal video game association (Aperture Science). Not immediately obvious meaning to non-technical audience.

4. PriorIQ

"Intelligent prior authorization."

Pros: Tech-forward, category-defining. Immediately communicates function. "IQ" implies intelligence/AI. Compact and memorable.

Risks: Narrow — sounds like it only does PA (which is the intent, but limits future expansion). May feel cold.

5. SwiftAuth

"Authorization at the speed of care."

Pros: Speed + authorization. Direct and action-oriented. Easy to remember. Clean URL availability likely.

Risks: Sounds like a cybersecurity/authentication company (OAuth, AuthO). May confuse the market.

Recommendation: GreenLight by Medivis is the strongest choice. The "green light" metaphor is universally understood, emotionally positive, and directly maps to the core value proposition — getting authorization approvals. The "by Medivis" parent brand adds enterprise credibility and positions the platform within a broader healthcare technology portfolio. It works as both a noun ("the GreenLight platform") and a verb ("GreenLight your imaging orders"), giving it natural virality in clinical conversations.

APPENDIX A1

Third-Party Market Validation & PA Sizing

Independent research data from CAQH, KFF, AMA, IDC, and market analysts confirming the prior authorization market opportunity.

Total PA Market Size — Multiple Sources

METRIC	ESTIMATE	SOURCE
Total annual PA admin waste	$41.4-55.8B	IDC (2025)
Provider-side PA spend	$31B/yr	Valer Health
PA automation platform market (2024)	$2.18B	Growth Market Reports
PA automation market projected (2033)	$11.32B	GMR (18.4% CAGR)
FHIR-based PA market (2025)	$745M	Future Market Insights
FHIR PA market projected (2036)	$3.2B	Future Market Insights
CMS projected savings (10-yr)	$15B	CMS / AMA

PA Volume Breakdown by Payer Segment

SEGMENT	VOLUME/YEAR
Medicare Advantage	53M
Traditional Medicare	625K
Commercial (medical)	~182M
Medicaid managed care	~80-120M
Total medical PAs	**~350-500M**

Sources: KFF (2024), CAQH Index (2025), AMA Physician Survey (2024). Pharmacy PAs add an estimated 150–200M+ additional transactions via CoverMyMeds/Surescripts.

39
PAS PER PHYSICIAN PER WEEK (AMA 2024, N=1,000)

13 hrs/wk
PHYSICIAN + STAFF TIME ON PA (AMA SURVEY)

100K+ RNs
EQUIVALENT FTE RN TIME CONSUMED BY PA ANNUALLY (AMA)

43% ↑
INCREASE IN PRACTICE PA STAFFING SPEND, 2019–2024 (MGMA)

Processing Cost Per PA Transaction

METHOD	PROVIDER COST	PAYER COST	COMBINED
Manual (phone/fax)	$10.97	$3.52	$14.49
Portal/partial electronic	~$7.19	~$2.00	~$10.00
Fully electronic	$1.88-5.79	$0.05	~$2-6

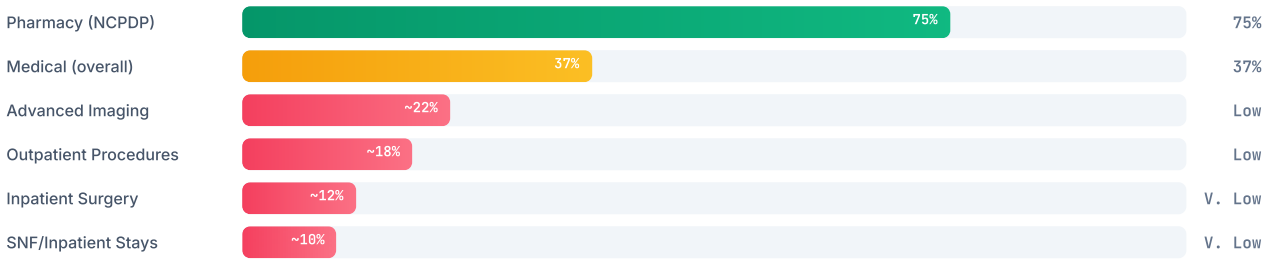
Sources: CAQH 2025 Index, 4sight Health, Health Affairs Scholar. All-in estimates including overhead: $40–50/PA for payers, $20–30/PA for providers.

APPENDIX A2

The Automation Gap: Why Imaging & Surgery Lag

Independent data confirms that imaging, surgical, and procedural PAs are the least automated and most manually intensive segment of prior authorization.

Electronic PA Adoption by Category



Sources: CAQH 2024–2025 Index (35–40% overall medical electronic rate); AMA (45% of all PA requests still submitted by fax). Subcategory estimates are directional based on industry analysis. Surescripts pharmacy ePA covers 84% of prescribers.

Why Imaging/Surgical PAs Resist Automation

Unstructured Clinical Documentation

Requires narrative clinical notes, prior imaging results, failed conservative treatment docs, and lab values — scattered across EHR systems. Pharmacy PAs use structured formulary data.

Multiple RBM Gatekeepers

EviCore, Carelon, and NIA each have different portals, rules, and workflows. Payers switch RBMs without notice — breaking provider workflows overnight.

Payer Criteria Variability

Each payer uses different criteria (InterQual, MCG, proprietary). Same MRI approved by one insurer, denied by another for identical patients. No standardization.

PACS/RIS Integration Needed

Imaging PAs require connection to radiology-specific systems, not just EHRs. Most horizontal PA companies don't have this capability.

Peer-to-Peer Review Burden

Unique to medical/surgical PAs. Physicians must schedule phone calls with insurance medical directors. ASTRO documents reviewers lacking relevant specialty expertise.

Scheduling Cascade Impact

Delayed surgical PA disrupts OR schedules, anesthesia, patient prep, and downstream appointments — a multi-party coordination problem pharmacy PA doesn't face.

The damning statistic: 80.7% of appealed Medicare Advantage PA denials were overturned in 2024 (KFF). Some insurers exceed 95% overturn rates. But only 11.5% of denials are actually appealed. The system works as a friction-based cost reduction mechanism — most denials are effectively wrong, but payers profit from providers who don't fight back. **An AI that gets submissions right the first time attacks this directly.**

APPENDIX A3

Revenue Per PA: The Value Hierarchy

A surgical PA protects 100x more revenue than a pharmacy PA. This is why the imaging/surgery niche commands premium economics.

Revenue Protected Per Prior Authorization by Type

PA CATEGORY	REVENUE AT STAKE	RELATIVE VALUE	AUTOMATION STATUS
Standard pharmacy	$15–$500	1x	Solved
Specialty pharmacy	$2K–$15K/mo	10–30x	Partial
Advanced imaging (MRI/CT/PET)	**$500–$4,000**	**3–10x**	Broken
Outpatient procedures	**$1K–$15K**	**10–30x**	Broken
Inpatient surgery	**$10K–$100K+**	**100–200x**	Broken
DME	$500–$30K	5–60x	Partial
Part B drugs (infusions)	$2K–$20K+	20–40x	Partial

Sources: Medicare fee schedules, Johns Hopkins hospital radiology pricing study, Plancher Orthopedics cost analysis, PMC commercial imaging price variation study.

Specific Claim Values in GreenLight Target Segments

$29,300
AVERAGE TOTAL KNEE REPLACEMENT CLAIM (RANGE: $15K–$70K)

$50K+
LUMBAR FUSION TOTAL CLAIM VALUE

$1,788
BRAIN MRI WITH CONTRAST (COMMERCIAL RATE)

$1,311
LUMBAR MRI (COMMERCIAL RATE VS. $269 MEDICARE)

Provider Pain Points — By the Numbers

- **94%** of physicians say PA delays access to necessary care (AMA 2024)
- **29%** report serious adverse events from PA delays (hospitalization, impairment, death)
- **82%** report PA leads to treatment abandonment by patients
- **87%** say PA leads to *higher* overall healthcare utilization — the opposite of its intent
- **92%** of medical groups have hired staff exclusively for PA (MGMA)
- **$83K/yr** per radiology practice in PA-related costs (14 hrs/week radiologist time)
- **$1.5M+/yr** lost by mid-sized radiology centers to PA denials and delays
- **96%** of AAHKS surgeons report PA delays timely surgical access
- In radiation oncology: **7%** of surveyed oncologists linked PA directly to patient deaths

The economic math: A single denied surgical PA puts $15K–$50K+ in provider revenue at risk. A denied pharmacy PA puts $15–$500 at risk. Preventing one surgical denial recovers as much revenue as preventing 100 pharmacy denials. This fundamentally changes willingness to pay for automation — and explains why **$8/PA** is a compelling price point for imaging and surgical practices.

APPENDIX A4

CMS-0057-F: The Federal Mandate & API Layer

The CMS Interoperability and Prior Authorization Final Rule (89 FR 8758, published Feb 8, 2024) creates mandatory FHIR-based PA infrastructure for the exact categories GreenLight targets.

What CMS-0057-F Requires

REQUIREMENT	DESCRIPTION	IMPACT
Prior Authorization API	FHIR-based API for providers to check PA requirements, discover documentation needs, submit requests, and receive decisions electronically	Core enabler
Decision Timeframes	72 hours for urgent/expedited; 7 calendar days for standard (down from 14 days)	Faster cycles
Specific Denial Reasons	All denials must include specific reasons regardless of submission method	Appeal fuel
Public Metrics Reporting	Payers must publish approval rates, denial rates, appeal outcomes, turnaround times annually	Transparency
Provider Access API	Payers share claims, encounters, and PA info with in-network providers	Data access

Compliance Timeline — Hard Deadlines

Jan 1, 2026 — NOW IN EFFECT

72-hour/7-day decision timeframes active. Specific denial reasons required. PA metrics collection begins for all impacted payers.

Operational

March 31, 2026 — IMMINENT

First public reporting of CY2025 PA metrics. Payer denial rates become visible for the first time.

Transparency

Jan 1, 2027 — THE BIG ONE

Prior Authorization API must be live for all MA organizations, Medicaid/CHIP FFS, managed care, and QHP issuers. FHIR-based electronic PA becomes mandatory.

API mandate

2028 — Full Enforcement

States may receive one-year extension for Medicaid/CHIP. Enhanced federal financial participation (FFP) for implementation. Full ecosystem operational.

Universal

Critical scope detail: CMS-0057 covers **medical items and services** but **explicitly excludes pharmacy benefit drugs**. This means the rule directly mandates electronic infrastructure for imaging, surgery, and procedures — while leaving pharmacy PA to existing CoverMyMeds/Surescripts infrastructure. The rule creates the plumbing GreenLight connects to.

FHIR-only enforcement discretion: CMS waived HIPAA X12 278 enforcement for payers implementing all-FHIR PA APIs (Feb 28, 2024). New entrants don't need legacy EDI capabilities. The Da Vinci PAS, CRD, and DTR implementation guides are recommended but not mandated — creating flexibility for innovators.

Related Federal Legislation

Improving Seniors' Timely Access to Care Act (H.R. 3514 / S. 1816, 119th Congress, introduced May 2025). Would codify electronic PA requirements in statute, add real-time decisions for routinely approved items. Strong bipartisan support but has failed in multiple prior sessions. GovTrack gives <1% chance of passage.

Political risk is low: The current administration suspended some MA transparency requirements but has **not touched CMS-0057-F**. The rule enjoys broad bipartisan support (reducing admin burden is popular across the spectrum). 67% of health plans plan to reevaluate PA vendor contracts by 2026.

APPENDIX A5

Strategic Assessment: Bull Case vs. Bear Case

An honest, data-driven analysis of whether the imaging/surgery PA niche is worth a multi-year commitment.

The Bull Case

1. Massive, Proven Market

$35–56B in annual waste. $1.5–30M revenue leakage per facility. 94% of physicians say PA harms care. Pain is real and quantified.

2. Clear Competitive Whitespace

CoverMyMeds solved pharmacy PA. No equivalent exists for medical/imaging/surgical PA. The "CoverMyMeds for medical PA" position is genuinely available.

3. Regulatory Tailwind with Hard Deadline

CMS-0057 forces payers to build FHIR APIs by Jan 2027 — creating standardized infrastructure that GreenLight builds on. Timing is excellent.

4. Complexity = Defensibility

Unstructured clinical docs, payer variability, RBM fragmentation, PACS integration create genuine moats. Rules engine compounds over time with more PA outcomes data.

5. Unit Economics Work

$8/PA with 3–4x customer ROI. Performance guarantee de-risks adoption. Revenue per customer scales from $77K/yr (imaging center) to $4.8M/yr (multi-hospital).

6. Validated by Market Signals

Cohere Health ($5.5B valuation) proves PA AI is valued. 94% of payers plan substantial AI-PA investment. 67% plan to reevaluate vendors by 2026. Demand is surging.

The Bear Case & Mitigations

1. EHR Vendors Build Native PA

Epic already has some PA features. **Mitigation:** EHR tools will be generic; imaging-specific intelligence (ACR criteria, PACS/RIS, RBM navigation) is too specialized for EHR platforms. Partner, don't compete.

2. Olive AI Cautionary Tale

$400M raised at $4B valuation, collapsed in 2023. **Mitigation:** Olive failed from lack of focus ("automate everything"). GreenLight's vertical niche strategy is the opposite approach.

3. CMS-0057 Could Reduce PA Volume

Faster decisions + transparency may eliminate some PAs. **Mitigation:** MA PA volume has actually grown from 37M (2019) to 53M (2024) despite reform talk. Payers add PAs faster than they remove them.

4. Customer Concentration Risk

15 customers in Year 1 creates dependency. **Mitigation:** GTM plan diversifies across facility types (imaging centers → ASCs → hospitals). Enforce no single customer >15% of revenue.

5. Payer Portal Instability

Payers change portals, rules, and RBMs without notice. **Mitigation:** CMS-0057 API mandate reduces portal dependency. Dedicated monitoring team required as permanent OpEx.

VERDICT

This is not a mirage. The convergence of massive pain ($35B+ waste), clear whitespace (no CoverMyMeds for medical PA), and a regulatory catalyst (CMS-0057, Jan 2027) makes this a rare setup. The key risk is execution, not market validity. Worth going all-in.